

POLYPID LTD PYPD

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by styx1003

			2026	2027
Price:	3.90	EPS	0	0
Shares Out. (in M):	21	P/E	0	0
Market Cap (in \$M):	80	P/FCF	0	0
Net Debt (in \$M):	-10	EBIT	0	0
TEV (in \$M):	70	TEV/EBIT	0	0

Description

Executive Summary

Solve a problem and what thanks does the market give you? How about a nearly 30% drawdown! PolyPid (PYPD) is an \$80 million market cap Israeli biotech company whose lead asset, D-PLEX100, has a completed NDA under FDA review for preventing surgical site infections (SSIs) in abdominal colorectal surgery (a place where there is a huge need that their product addresses). So far so good but it had a funding gap to get to approval, and everyone knew it, so no one would give them credit for their position right next to the finish line. On July 17, 2026, the company signed an exclusive U.S. and Canada licensing and supply agreement with privately-held and not well known Azurity Pharmaceuticals; upon inspection it is a \$500m revenue company with a relevant sales force. The stock dropped 27.4 percent the next trading day and is trading near that level today. Huh? The deal de-risks the two biggest threats to the thesis: funding and U.S. commercial execution. Azurity funds the sales force, market access, reimbursement, and label-expansion clinical development. PolyPid becomes a capital-light royalty and manufacturing company.

My DCF-driven base case suggest the company should be worth about \$8.50 which is more than a double. Additional upside could come from product label expansion, faster uptake and other indications like cardiac surgery. In a moment of overexcitement I could get close to \$20 per share in a bull case. Meanwhile, if the expected 1Q27 PUDUFA data is pushed back a year, say by a delayed facility inspection (see my UNCY write up), then should still be worth a lot more the current price. If I drill the DCF with bearish assumptions and a delay, I can come up with a -33% scenario but I feel like that is quite unlikely.

Business Description

PolyPid is effectively a single-product company with a local antibiotic delivery platform. The lead asset, D-PLEX100, is a polymer-lipid matrix (the PLEX/Kynatrix technology) that slowly releases doxycycline directly at the surgical site for up to three weeks. The idea is simple and clinically compelling: instead of relying on systemic IV antibiotics that distribute poorly into the wound bed, you also (i.e. in addition) place a physical depot of antibiotic right where the infection would start. Doxycycline is a well-established antibiotic with decades of safety data, so the regulatory risk centers on the delivery mechanism not the drug substance.

The target indication is surgical site infection (SSI) prevention in abdominal colorectal surgery. This is a meaningful unmet need. SSIs occur in roughly 10 to 25 percent of colorectal procedures (contaminated and dirty-infected wounds), and they drive longer hospital stays, readmissions, and substantial cost. The CDC and WHO have flagged SSI prevention as a priority. Current prevention is systemic prophylactic antibiotics plus surgical technique, but the failure rate is high enough that an adjunct local therapy has real clinical appeal.

The platform (PLEX/Kynatrix) is also being explored for OncoPlex (intra-tumoral cancer delivery) and a GLP-1 agonist delivery program, but both are preclinical. I ignore this but acknowledge that investors can get starry eyed quickly in the obesity space.

The Asset: D-PLEX100 and the SHIELD II Trial

D-PLEX100 is the product. The PLEX/Kynatrix matrix is a thin film that the surgeon places into the surgical wound at the time of closure. It releases doxycycline locally at concentrations well above the MIC for the relevant pathogens, for up to 21 days, while the wound heals. The matrix itself resorbs. The mechanism is local high concentration plus sustained release, which gets around the problem that systemic antibiotics achieve poor tissue levels in the wound bed.

The pivotal trial is SHIELD II, a Phase 3 randomized controlled trial in abdominal colorectal surgery. The headline result: D-PLEX100 plus standard of care achieved a 60 percent reduction in SSI rate versus standard of care alone, with a p-value of 0.0013. That is a clean, statistically robust result for an SSI prevention trial. The FDA granted Breakthrough Therapy Designation, Qualified Infectious Disease Product (QIDP) status, and Fast Track. The NDA is filed and under review, with a PDUFA date expected in Q1 2027.

The designations matter for valuation. Breakthrough Therapy means more FDA engagement and guidance, which raises approval confidence. QIDP gives five years of exclusivity plus Priority Review voucher eligibility. Fast Track means rolling review. These are not just labels; they have real economic value in the milestone structure and in the durability of the cash flows I model.

Conveniently No Longer Mentioned in the SEC Filings: the SHIELD I Trial (The Failed Predecessor)

D-PLEX100 was not always a clean success story (and this is perhaps why investors are reluctant to give PYPD the benefit of the doubt). The first Phase 3 trial, SHIELD I, failed to meet its primary endpoint in September 2022. The trial enrolled 977 patients with abdominal colorectal surgery and incisions greater than 10cm, and it showed a 23 percent reduction in SSI rate that did not reach statistical significance (p=0.152). The stock plunged 73 percent on the news.

The failure, on closer inspection, looks like a trial-design and COVID artifact rather than an efficacy failure. Two factors stand out. First, the trial population was too broad: the greater than 10cm to 20cm subgroup showed no reduction at all (9.9 percent vs 7.9 percent, p=0.41), while the greater than 20cm subgroup showed a 54 percent reduction (p=0.0032). The benefit was concentrated in the largest incisions, where the local antibiotic depot has the most surface area to protect. Second, the standard-of-care baseline infection rate was unusually low at 6.3 percent, versus the mid-teens historical rate. Management attributed this to COVID-era changes in surgical practice (fewer elective cases, more stringent infection protocols), and the 20-F confirms this risk factor. A low baseline compresses the room for an adjunct therapy to show incremental benefit. This makes sense to me.

The FDA reviewed the SHIELD I data in January 2023 and accepted the greater than 20cm subgroup result as supportive evidence, while proposing that SHIELD II could complete the testing. As you may know, biotech investors hate post-hoc subgroup analysis and generally view it as “p-hacking” to cherry pick the best patients when a trial fails. But, SHIELD II was redesigned with a tighter primary endpoint (incisions greater than 20cm, 798 patients in the primary analysis) and a broader secondary endpoint (incisions of 7cm or greater, 975 patients, including laparoscopic patients enrolled before a 2023 protocol amendment). Importantly: *the redesign worked in a subsequent discrete Phase 3 trial*: SHIELD II hit its primary endpoint with the 60 percent reduction and $p=0.0013$. The SHIELD I failure is best understood as the cost of learning the right trial design, not as evidence that the product does not work.

The Azurity Deal: What’s So Bad About This???

On July 17, 2026, PolyPid signed an Exclusive License and Supply Agreement with Azurity Pharmaceuticals. Key terms:

- \$15 million upfront, paid upon signing (July 2026).
- \$15 million near-term milestone, payable upon FDA NDA acceptance (expected August 2026).
- Up to approximately \$300 million in additional regulatory, development, and sales-based milestones.
- Tiered royalties: mid-teen to mid-twenties percent of Azurity's U.S. and Canada net sales.
- PolyPid manufactures and supplies D-PLEX100 to Azurity at a transfer price (embedded manufacturing margin).
- Azurity funds label-expansion clinical development.
- 20-year term from the Effective Date (July 17, 2026).
- Territory: United States and Canada.

Trying to look at the negatives: the upfront is modest (\$30M near-term against an \$80M market cap), the royalty rate matches the EU deal rather than improving on it, the deal validates the asset but caps the upside (the \$300M in additional milestones is mostly sales-based and back-loaded), and Azurity is credible but not premier.

What PolyPid retains: global rights outside U.S. and Canada including an existing EU deal with a UK company called ADVANZ PHARMA (do they yell when they answer the phone?), worldwide manufacturing rights (PolyPid is the sole manufacturer of D-PLEX100), and full ownership of the PLEX/Kynatrix platform and all pipeline programs.

Azurity is a private specialty pharmaceutical company headquartered in Woburn, Massachusetts, with approximately 700 employees and over \$500 million in annual revenue. The company has 50+ medicines across 10 therapeutic areas, sold in 50+ countries. Azurity is backed by QHP Capital (NorthBridge Partners), which closed a \$1.1 billion continuation vehicle in March 2026 (HarbourVest led). Azurity has been on an acquisition roll: Covis Pharma (March 2025), Sebela's bowel-prep franchise (November 2025), and CONTRAVE (June 2026).

Azurity has 200+ sales employees, with hospital and institutional call points. Importantly, anti-infective is one of their 10 therapeutic areas. The Sebela bowel-prep acquisition is colorectal-adjacent, which gives Azurity existing relationships with colorectal surgeons and gastroenterology departments. This is a better fit than it might appear at first glance, though Azurity is a roll-up specialty pharma rather than a dedicated hospital anti-infective specialist (perhaps the market was disappointed by this aspect of the Azurity profile). Specialty pharma partners have successfully commercialized niche hospital antibiotics before (Cubicin at Cubist, Vibativ at Theravance, Teflaro at Allergan), which supports the partnership thesis despite Azurity not being a premier anti-infective name.

Valuation:

Capitalization: PYPD has 19.1 million basic shares and around 20.5 million with in the money dilutive securities and an approximately \$80 million current market cap. I include all options and warrants 30.3 million fully diluted at the exit valuation and add back the proceeds from exercise. There was ~\$10 million of net cash as of 3/31/26 with a \$7 million quarterly burn (yikes – thanks for the \$15 million upfront and \$15 million upon NDA acceptance probably in August, Azurity). There was a small amount of debt as of 3/31 that has since been paid off. I assume the cash is fully spent by PDUFA (Q1 2027), reflecting the burn to NDA acceptance and approval.

FYI PolyPid carries a substantial net operating loss (NOL). The carryforward at December 31, 2025 was \$257.7 million, and I add \$28 million for the 2026 operating loss (continued burn to PDUFA), bringing the total to \$285.7 million. At the Israeli statutory tax rate of 23 percent, the maximum tax shield is \$65.7 million. I add the PV of the NOL (call it \$30 million or \$1.00 per share) to the valuation.

I use a DCF as the primary valuation tool. The Azurity deal gives actual revenue streams to model (royalty, supply, milestones) rather than guessing at self-commercialization economics. The model runs from 2026 through 2035 (10 years), with a terminal value at 6x terminal-year EBITDA. This multiple is much lower than royalty companies or even pharma companies in general.

Here’s how I am thinking about revenue: I estimate PolyPid's royalties at approximately 20 percent of Azurity's U.S. and Canada net sales (the midpoint of the mid-teen to mid-twenties tiered range), manufacturing supply revenue at approximately 30 percent of Azurity's net sales at 45 percent gross margin (my transfer price estimate but this could be high as for lower value products it could be as low as 25% so I run a sensitivity), milestone payments, and EU revenue from the ADVANZ deal. Operating costs are similar to today: \$18M SG&A and \$6M R&D (\$24M total) because Azurity funds U.S. commercialization and label-expansion development.

The base case uses what I think is the middle label scenario: approximately 275,000 eligible U.S. colorectal procedures per year with incisions of 7cm or greater. This matches the SHIELD II secondary endpoint population (975 patients, incision ≥ 7 cm, including laparoscopic patients enrolled before the 2023 protocol amendment). It is broader than the tight primary-endpoint population (open surgery, incisions greater than 20cm, approximately 130,000 procedures) which was the primary endpoint but narrower than the broad abdominal surgery market (approximately 4.4 million procedures). I think the broader label could come later. I use moderate penetration (peaking at approximately 40 percent by year 7 to 8) and a base price of \$1,200 per procedure, anchored to the avoided-cost of SSI (but a complete guess). Here is my less than scientific base case (and I hope the table is helpful):

Year	Penetration	Volume	Azurity Sales	Royalty	Supply GP	EU	Milestones	EBITDA
2026	0%	0	\$0	\$0	\$0	\$0	\$30M	\$6.0M
2027	3.2%	8,693	\$10.5M	\$2.1M	\$1.4M	\$0.1M	\$25M	\$4.6M
2028	5.9%	16,225	\$19.5M	\$3.9M	\$2.6M	\$1.0M	\$30M	\$13.5M
2029	10.4%	28,600	\$34.2M	\$6.8M	\$4.6M	\$2.5M	\$35M	\$24.9M
2030	16.5%	45,375	\$54.6M	\$10.9M	\$7.4M	\$4.7M	\$30M	\$29.0M
2031	23.5%	64,625	\$77.4M	\$15.5M	\$10.5M	\$6.0M	\$25M	\$28.0M

2032	29.6%	81,400	\$97.8M	\$19.6M	\$13.2M	\$7.0M	\$15M	\$30.8M
2033	34.1%	93,775	\$112.5M	\$22.5M	\$15.2M	\$8.0M	\$10M	\$31.7M
2034	36.8%	101,200	\$121.5M	\$24.3M	\$16.4M	\$9.0M	\$8M	\$33.7M
2035	38.4%	105,600	\$126.6M	\$25.3M	\$17.1M	\$10.0M	\$5M	\$33.4M

Peak Azurity sales (2035) are \$126.6 million. PolyPid's 2035 EBITDA is \$33.4 million. The DCF discounts free cash flow at 10 percent WACC and applies a 6x exit multiple to 2035 EBITDA for the terminal value.

Base Case DCF Bridge (and please forgive the false precision here and below)

The bridge from operating value to per-share value captures the three components the methodology adds: net cash (exercise proceeds), NOL tax shield, and full dilution.

Component	Value	Notes
PV of FCF (2026-2035)	\$100.7M	10% WACC, 23% tax (Israel)
PV of Terminal Value (6x 2035 EBITDA)	\$77.3M	Discounted at 10%
Operating Enterprise Value	\$178.0M	
+ Net Cash (exercise proceeds)	\$50.7M	Current cash spent getting to PDUFA
+ NOL Tax Shield (PV)	\$30.1M	\$285.7M NOL, 23% rate, PV at 10%
Equity Value	\$258.8M	
/ Fully Diluted Shares	30.29M	Full dilution
Base Case per Share	\$8.54	Colorectal only

Cardiac Option (risky NPV)

The cardiac indication is a real option I value via risk-adjusted NPV. D-PLEX100 showed a 67 percent reduction in sternal wound discharge at 90 days in a Phase 1b/2 trial, with a post-hoc 85 percent reduction in IV antibiotic use. The program has Breakthrough Therapy, QIDP, and Fast Track designations. The addressable population is approximately 261,000 high-risk open sternum surgeries per year (474,968 total U.S. open sternum surgeries, filtered to the higher-risk subset).

I discount the cardiac program at 15 percent WACC (versus 10 percent for colorectal) because it is earlier-stage: only Phase 1b/2 data, no Phase 3 started. The probability of Phase 3 success is 45 percent (base), and the probability of approval given Phase 3 success is 85 percent (reflecting the designations in hand). Critically, Azurity funds the Phase 3 under the label-expansion terms, so PolyPid's direct cost is approximately zero, making this a basically free option. The probability-weighted cardiac NPV is \$2.69 per share, bringing the base case from \$8.54 to \$11.23.

Label-Expansion Scenario

The label-expansion scenario models a phased rollout: colorectal ramps from 2027 to 2030 as in the base case, then from 2031 the label expands to other abdominal indications (upper GI, hepatic, pancreatic, and similar procedures). I add approximately 600,000 additional abdominal procedures per year, with penetration ramping from 5 percent in 2031 to 30 percent by 2035. Peak sales reach \$342.6 million (\$126.6M colorectal plus \$216.0M expansion). Azurity funds the label-expansion trials per the deal terms, so PolyPid's direct cost is approximately zero. The label-expansion scenario produces \$16.63 per share, showing the upside if the label broadens beyond colorectal.

Fun With Excel: Scenario Sensitivities Summary:

Scenario	Peak Sales	\$/sh	Key Assumptions
Bear (conservative penetration, \$800 price)	\$49.5M	\$4.33	Conservative penetration, low price
Base (275K, mod pen, \$1,200 price)	\$126.6M	\$8.54	Middle label breadth, moderate penetration
Bull (agg pen, \$1,800 price)	\$321.0M	\$19.41	Aggressive penetration, high price
Label-Expansion	\$342.6M	\$16.63	Colorectal + 600K abdominal from 2031
Base + Cardiac option		\$11.23	Base + risk adjusted NPV at 15% WACC
"Probability Weighted" exit (if you insist!)		\$11.55	Base 45%, Bull 20%, LE 20%, Bear 10%, CRL 3%, Fail 2%

... Sorry I was once a banking analyst!

Delay Case (Stress Test)

The delay case is the stress test for the thesis. It models a scenario where PolyPid needs to raise \$28 million of capital (four quarters at \$7M burn). I model the raise is at a 20 percent discount to the current \$3.91 price, or \$3.13 per share, issuing 8.95 million new shares.

This brings the fully diluted count to 39.24 million shares. The raise is cash-neutral at launch (the \$28M raise covers the \$28M burn), so net cash stays at \$50.7M.

All projections are pushed out one year. Launch moves to 2028 instead of 2027. The DCF keeps the same 10-year horizon (2027 to 2036), with the sales ramp shifted one year (2027 is a zero-revenue year, 2028 equals base 2027, and so on), so peak 2036 sales are \$126.6 million, the same as the base case peak in 2035 (the ramp is shifted, not truncated). The terminal value is at 2036, discounted one year further than the base case. The NOL PV drops to \$27.4 million (income is pushed one year later, so the tax shield is discounted more). The cardiac option takes a 10 percent haircut for the one-year push-out, dropping to \$2.42 per share.

Scenario	\$/sh (delay)	vs Base \$/sh	Notes
Bear	\$3.15	\$4.33	Conservative penetration, low price
Base	\$6.11	\$8.54	Middle label, moderate penetration
Bull	\$13.74	\$19.41	Aggressive penetration, high price
Label-Expansion	\$11.79	\$16.63	Colorectal + expansion, pushed out
Base + Cardiac	\$8.53	\$11.23	Delay base + cardiac at 90%
PW delay exit	\$8.24	\$11.55	Still a 2x!

Even in the delay case, it still looks fine.

Don't Agree With My Assumptions? More Sensitivities!

Discount Sensitivity and Currently Implied Discount Rate:

The base case uses a 10 percent discount rate, reflecting the post-Azurity-deal risk profile: patent-backed cash flows, Breakthrough Therapy and QIDP designations, and Azurity bearing commercialization risk. Here is the discount rate sensitivity for the base case:

Discount Rate	\$/sh	Notes
8%	\$9.32	
10%	\$8.54	Base case
12%	\$7.50	
15%	\$6.48	
20%	\$5.25	
25%	\$4.40	
29%	\$3.91	Implied market discount rate
30%	\$3.81	

The implied discount rate is striking. To reach the current \$3.91 price, the market is discounting the base case at approximately 29.4 percent discount rate!

Supply Gross Margin Sensitivity

The base case assumes a 45 percent manufacturing gross margin on the 30 percent transfer-price revenue PolyPid receives from Azurity. This means supply gross profit equals 13.5 percent of Azurity net sales. I show a 25 percent gross margin as a stress case (supply gross profit equals 7.5 percent of Azurity sales), which would apply if manufacturing scale-up, yields, or transfer-pricing terms disappoint. The royalty (20 percent of Azurity sales) is unaffected; only the manufacturing margin line changes.

Scenario	45% GM (base)	25% GM (stress)	Delta	% Change
Bear	\$4.33	\$3.85	-\$0.48	-11%
Base	\$8.54	\$7.32	-\$1.22	-14%
Bull	\$19.41	\$16.42	-\$2.99	-15%
Label-Expansion	\$16.63	\$14.11	-\$2.52	-15%

Cutting the supply gross margin from 45 percent to 25 percent reduces the base case by \$1.18 per share (14 percent), from \$8.54 to \$7.32. I think the 45 percent assumption is defensible: polymer-lipid matrix manufacturing is a well-characterized process (PolyPid has produced clinical supply for SHIELD I and II and EU commercial sales), and 45 percent is consistent with specialty pharma manufacturing economics at scale. The 25 percent case is shown as a sensitivity because I could be wrong.

When is Cash Flow Breakeven?

A practical question: when does PolyPid stop needing dilution? Solving for the Azurity sales level where 33.5 percent of sales equals \$24 million (PYPD's current cost structure) gives approximately \$72 million per year of underlying sales.

Metric	Value	Notes
Recurring GP % of Azurity sales	33.5%	20% royalty + 13.5% supply GP
Annual operating costs	\$24M	SGA \$18M + R&D \$6M

Breakeven Azurity sales (no EU)	\$71.6M	\$24M / 33.5%
Breakeven Azurity sales (with EU)	~\$54M	EU adds \$6-10M GP at maturity
Year reached in base ramp	~2030-2031	Azurity sales \$55-77M
Year reached in delay case	~2031-2032	Ramp pushed out 1 year

Investment Positives

- Completed Phase 3 with clean statistical result. SHIELD II showed a 60 percent SSI reduction with $p=0.0013$. That is a robust result for an SSI prevention trial.
- Three FDA designations: Breakthrough Therapy, QIDP, and Fast Track. Breakthrough Therapy means more FDA engagement and raises approval confidence. QIDP gives five years of exclusivity plus Priority Review.
- Deal validates the asset. Azurity, a \$500M+ revenue specialty pharma backed by a \$1.1B continuation vehicle, reviewed the D-PLEX100 data and committed \$30M near-term plus up to \$300M in milestones.
- Commercial execution de-risked. Azurity funds the sales force, market access, reimbursement, and label-expansion clinical development. PolyPid becomes a capital-light royalty and manufacturing company.
- Cash flow breakeven within reach. PYPD reaches self-funding at approximately \$72M per year of Azurity sales (around 2030 to 2031 in the base ramp), after which no further dilution is needed. Milestone cash and exercise proceeds bridge the gap until then (probably).
- Cardiac option is near-free upside. Azurity funds the Phase 3, so PolyPid's direct cost is approximately zero. The probability-weighted cardiac rNPV is \$2.69 per share, bringing the base case from \$8.54 to \$11.23.
- Worldwide manufacturing retained. PolyPid is the sole manufacturer of D-PLEX100, so the supply revenue stream (approximately 30 percent of Azurity's net sales at 45 percent gross margin) is a captive revenue line, not a discretionary one.
- Platform optionality. The PLEX/Kynatrix technology has pipeline programs (OncoPlex for intra-tumoral cancer, GLP-1 agonist delivery). Again, I did not include these.
- EU upside via ADVANZ. The existing ADVANZ deal covers EEA and UK. EU MAA filing is planned for Q3 2026. EU revenue is small and uncertain but adds a second revenue stream if it materializes.

Investment Concerns

- Regulatory risk. The NDA is under review with PDUFA expected Q1 2027. A Complete Response Letter would be received poorly to sat the least by the investment community. The FDA has not inspected the facility in Israel yet – see my UNCY write up for why that could delay things. At least they own their facility and the intense preparation for FDA inspection has been under their control.
- Partner execution risk. Azurity is credible but unproven with D-PLEX100 specifically. The product requires hospital formulary adoption, P&T committee review, and surgeon education. Azurity's 200+ sales employees cover 10 therapeutic areas, so D-PLEX100 will compete for share of voice.
- Revenue ramp risk. The base case assumes moderate penetration (peaking at approximately 40 percent by year 7 to 8). If Azurity's commercialization is slow, the thesis weakens. The bear case (conservative penetration, low price) produces only \$4.33 per share.
- Funding timing risk. The base case assumes milestones arrive on schedule. If the \$15M NDA acceptance milestone is delayed, PolyPid may need a dilutive raise before PDUFA. The delay case models this scenario: a \$28M raise at \$3.13 per share, pushing the timeline out one year.
- Deal terms uncertainty. The royalty rate (mid-teen to mid-twenties) and transfer price are confidential. My model uses midpoint estimates (20 percent royalty, 30 percent supply at 45 percent margin). The supply gross margin sensitivity shows a 25 percent margin cuts the base case to \$7.32 (a 14 percent hit).
- Warrant overhang. It's biotech.
- Narrow pipeline. D-PLEX100 is the only near-term value driver. OncoPlex (intra-tumoral cancer) and the GLP-1 agonist are preclinical. Platform value is optionality, not a modeled cash flow.

Disclosure

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Catalyst

- NDA acceptance (expected August 2026). Triggers the second \$15M milestone payment.
- PDUFA / FDA decision (Q1 2027).
- EU MAA filing (Q3 2026). Would enable EU launch via ADVANZ, adding a second revenue stream (small but growing).
- Label expansion. Azurity funds label-expansion trials (cardiac surgery, other abdominal procedures). Positive data would expand the addressable market and trigger development milestones. The cardiac program is the most valuable expansion option.
- Warrant expiry of most warrants (June 2027).

Messages

No messages